

Adverse events mainly were hypothyroidism, fatigue and skin rash with no grade 3/4 adverse events.

Conclusions: Our patients with dMMR endometrial cancers showed remarkable efficacy and long-term outcomes. It is mandatory to check for MMR in endometrial cancer patients to identify these subgroups who do very well.

Legal entity responsible for the study: The authors.

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400TIP

An open-label, multicenter, phase II study of trastuzumab deruxtecan (T-DXd) in patients (pts) with HER2-expressing solid tumors: DESTINY-PanTumor02 (DP-02) part 2

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Background: T-DXd is a HER2-directed antibody-drug conjugate that is approved in multiple countries for the treatment of HER2-positive and HER2-low breast cancer, HER2-positive gastric or gastroesophageal junction adenocarcinoma, and HER2-mutant non-small cell lung cancer (NSCLC). In April 2024, T-DXd received accelerated approval in the US for adult pts with unresectable or metastatic HER2-positive (immunohistochemistry [IHC] 3+) solid tumors that have progressed after prior systemic treatment and have no satisfactory alternative therapies. In Part 1 of the DP-02 study (N=267), T-DXd showed clinically meaningful investigator-assessed (INV) objective response rates (ORRs; 37.1% [95% CI 31.3, 43.2]) and INV median progression-free survival (PFS; 6.9 months [95% CI 5.6, 8.0]) in pretreated pts with HER2-expressing solid tumors, with the largest effect observed in pts with centrally confirmed HER2 IHC 3+ tumors (ORR 61.3% [95% CI 49.4, 72.4]; median PFS 11.9 months [95% CI 8.2, 13.0]); safety was consistent with the known profile. Here, we describe Part 2 of the DP-02 study.

Trial design: DP-02 (NCT04482309) is an open-label, multicenter, Phase II study evaluating T-DXd (5.4 mg/kg intravenously once every 3 weeks) in pts with HER2-expressing locally advanced or metastatic solid tumors that have progressed following prior systemic treatment or have no alternative treatment options. Part 2 will enroll ~40 pts in each of the following cohorts: A, any HER2 IHC 3+ tumor; B, any HER2 IHC 2+ / in situ hybridization—positive tumor; C, HER2 IHC 2+/1+ endometrial cancer; D, HER2 IHC 2+/1+ ovarian cancer; or E, HER2 IHC 2+/1+ cervical cancer. Pts with breast, gastric, or colorectal cancer will not be enrolled; unlike in Part 1, pts with NSCLC will be eligible for cohorts A and B. HER2 expression will be determined by central testing and scored per ASCO/CAP gastric-specific criteria. The primary endpoint is INV confirmed ORR per RECIST 1.1. Secondary endpoints include duration of response, disease control rate, PFS, and overall survival. Safety, tolerability, pharmacokinetics, and immunogenicity will also be assessed.

Clinical trial identification: NCT04482309.

Legal entity responsible for the study: AstraZeneca and Daiichi Sankyo.

Funding: This study is sponsored by AstraZeneca. In March 2019, AstraZeneca entered into a global development and commercialization collaboration agreement with Daiichi Sankyo for trastuzumab deruxtecan (T-DXd; DS-8201).

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401TIP

Adebrelimab plus chemotherapy and bevacizumab induction therapy followed by maintenance therapy with adebreliamab plus fluzoparib and bevacizumab in platinum-sensitive relapsed ovarian cancer (CHANGCHUN): A single-arm, exploratory study

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Background: The phase II OPEB-01/APGOT-0V4 study (NCT04361370) suggested that triplet maintenance therapy of bevacizumab, olaparib, and pembrolizumab achieved durable efficacy and tolerable safety in patients with BRCA wild-type, platinum-sensitive recurrent ovarian cancer. Findings of the Phase III DUO-O trial (NCT03737643) indicated that induction chemotherapy plus bevacizumab with durvalumab, followed by triplet maintenance with durvalumab, bevacizumab, and olaparib could increase progression-free benefit in patients with newly diagnosed advanced ovarian cancer without tumor BRCA1/2 mutation. Two Phase II clinical trials, OPAL (NCT03574779) and GINECO BOLD (NCT04015739) implied that patients with platinum-resistant ovarian cancer may benefit from the combination of bevacizumab with an immune checkpoint inhibitor and a PARP inhibitor.

Trial design: The key inclusion criteria of this single-center, single-arm, prospective study are 1) 18-75 years female; 2) ECOG Performance Status score of 0-1; 3) pathologically confirmed FIGO staging III-IV ovarian epithelial cancer, epithelial fallopian tube cancer, or primary peritoneal cancer; 4) tumor recurrence $\geq$ 6 months after the last dose of platinum-containing chemotherapy; 5) germline BRCA wild-type; 6) no more than 1 prior PARP inhibitor. All patients will receive paclitaxel/carboplatin plus bevacizumab and adebreliamab (a PD-L1 monoclonal antibody) induction therapy followed by maintenance therapy with bevacizumab plus adebreliamab and fluzoparib (a PARP inhibitor) until disease progression, intolerable toxicity, withdrawal of informed consent, or investigator assessment that continuation is inappropriate. The primary endpoint is the 12-month progression-free survival rate. The secondary endpoints are overall survival, objective response rate, progress-free survival in patients with complete response/partial response/stable disease after induction chemotherapy, and safety. Patient recruitment was started on June 7, 2024.

Clinical trial identification: NCT06446206.

Legal entity responsible for the study: The authors.

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